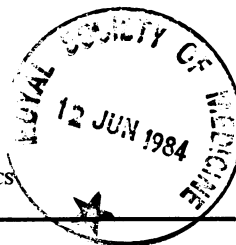


Clinical Section

President George Qvist FRCS



Meeting March 11 1966
at the new Academic Centre,
Whittington Hospital, London
(Continued from October 'Proceedings', p1006)

Cases

Malignant Melanoma: Regression of Metastases after Excision of Primary Growth
N E Stidolph BM FRCS
(Whittington Hospital, London)

R P, male, aged 23. Cutter and designer

History: First seen 4.1.62. Four months previously a 'spot' appeared on his left breast which he squeezed. The lump slowly increased in size, and a further tender swelling developed in the left axilla ten weeks later. He was given an antibiotic by his own doctor and shortly after this a 'rash' appeared over the left breast, axilla and scapula regions. On holiday the patient saw a doctor who said the 'rash' was due to the antibiotic and prescribed Anthisan.

On examination: Red-haired man. On the left breast 6 cm lateral and slightly above the nipple was a lesion 4 cm in diameter with surrounding skin black and indurated, the whole lesion fixed to the underlying tissues. The breast, axilla and scapula region were studded with small red papillary lesions and a large fixed node 3 cm in diameter was present in the left axilla.

A diagnosis of malignant melanoma was made.

Operation (8.1.62): Growth excised; skin nodules 5 cm from primary growth also excised for biopsy. The pathological report showed these to be non-pigmented melanomata primary and secondary.

Progress: By 18.1.62 the axillary node was much reduced in size and the satellite nodules were barely perceptible. On 25.1.62 he was referred for radiotherapy, and by 8.2.62 the whole area was cleared and the wound well healed.

When last seen on 16.9.65 he was fit and had no evidence of recurrence anywhere.

Mr H H G Eastcott said it was possible for melanomatous deposits to undergo spontaneous regression without any doubt, as in one elderly woman with

massive inguinal and deep iliac node metastasis, confirmed by exploration and biopsy, which the late Sir Gordon Gordon-Taylor and he had followed; the case had been thought unsuitable for hind-quarter amputation and no further active treatment had been given. Over the next twelve months the deposits had steadily regressed and eventually became almost impalpable, though there had remained some lymphoedema of the groin and of the limb which might have obscured them. About two and a half years later she had returned with a large mass in the opposite flank, and not long afterwards had died, most probably of melanomatous deposits, though no post-mortem examination could be obtained.

Mr George Qvist (*President*) recalled the case of a man aged 19 who had a mole removed from the skin of the back of the chest and who, three years later, developed secondary lymph nodes in the left axilla and in the right chest wall. These two groups of nodes had been excised eighteen years ago and followed by radiotherapy; there had been no further metastasis since.

Prepyloric and Pyloric Mucosal Stenosis (Four Cases)

S L Strange FRCS(Ed)
(Whittington Hospital, London)

Definition: A diaphragm, consisting of a double layer of mucosa, is present at the pylorus or less commonly in the prepyloric region. This diaphragm has an opening in it, which may be less than 2 mm in diameter.

Case 1 Prepyloric Mucosal Stenosis

Mrs M G, aged 49

History: 1961-63: Heartburn, nausea and occasional vomiting of food; no pain. November 1963: Repair operation for uterine prolapse. December 1963: Hæmatemesis which required blood transfusion.